

ECG computer-assisted analysis report

Generated 15 June 2026 — This document summarizes automated classifier output. It is not a substitute for professional medical diagnosis.

1. Patient & lab information

Patient: youssef amr | **National ID:** 30105012345678

Lab: Al Mokhtabar labs (Al Mokhtabar 123)

Lab address: Alexandria , 228 Port Said Street, Zamzam Tower - Sporting

ECG record ID: cmqerd2gf00031qk7227lvm9e | **ECG test date:** 2026-06-15T05:13:56.655Z

2. Primary finding

Primary label (model): Normal ECG (NORM)

Highest probability: 99.44%

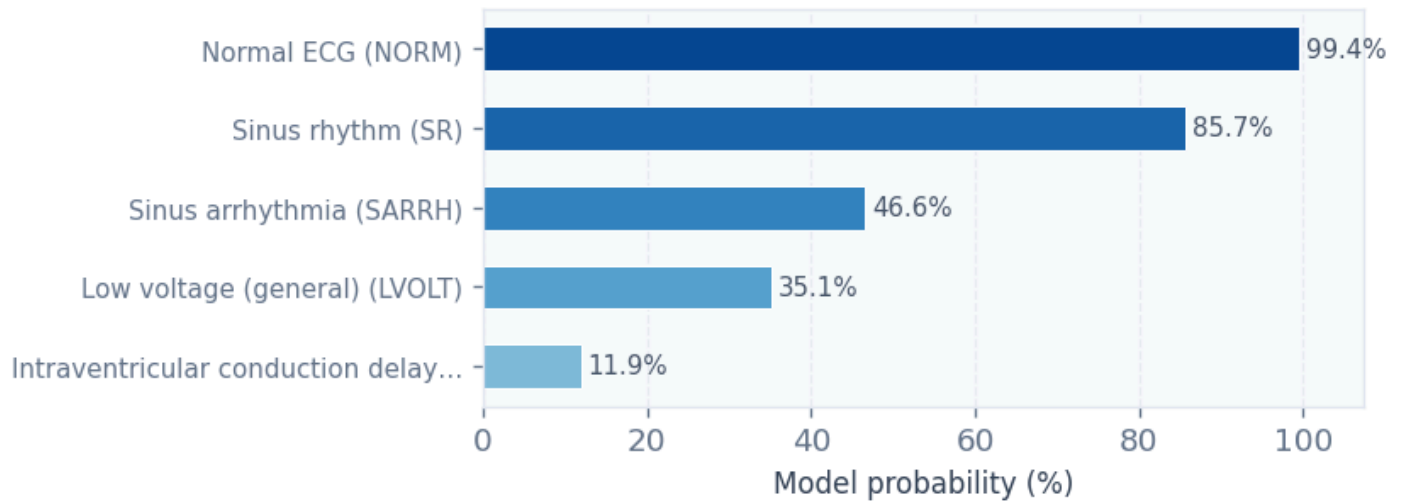
Confidence (qualitative): High model confidence (still not a clinical diagnosis)

3. Top five predicted statements

#	Label	Code	Probability (%)	Confidence note
1	Normal ECG (NORM)	NORM	99.44	High model output
2	Sinus rhythm (SR)	SR	85.74	High model output
3	Sinus arrhythmia (SARRH)	SARRH	46.60	Limited
4	Low voltage (general) (LVOLT)	LVOLT	35.09	Limited
5	Intraventricular conduction delay (IVCD)	IVCD	11.92	Low

4. Top five visualization

Top 5 ECG findings (PTB-XL-style labels)



5. Medical interpretation (AI-assisted narrative)

The automated ECG classifier results may suggest a normal ECG pattern, but it is essential to correlate these findings with clinical history, vitals, and physical examination to determine the overall clinical significance. The presence of sinus rhythm, sinus arrhythmia, low voltage, and intraventricular conduction delay patterns could be consistent with various cardiac conditions, but these findings are not definitive diagnoses. It is crucial to consider that automated classifiers can be wrong due to factors such as electrode placement, artifact, and patient-specific characteristics.

Urgency: If you are experiencing symptoms such as chest pain, syncope, or severe dyspnea, it is recommended that you seek immediate medical attention at an emergency care facility. For non-urgent concerns, it is advised to discuss these results with a qualified clinician during a routine follow-up appointment to determine the best course of action.

Follow-up: It is recommended that you follow up with your primary care physician or a cardiologist to discuss these results in the context of your overall health and medical history. They will help determine the need for further evaluation, testing, or referral to a specialist.

6. Warning signs to seek urgent care

- Chest pain or discomfort
- Syncope or near-syncope
- Severe dyspnea or shortness of breath
- Palpitations or irregular heartbeat
- Dizziness or lightheadedness

7. Recommendations

- Maintain a healthy lifestyle, including a balanced diet and regular exercise
- Monitor and manage any underlying medical conditions, such as hypertension or diabetes
- Attend routine follow-up appointments with your healthcare provider
- Discuss any concerns or symptoms with your clinician promptly
- Consider keeping a symptom journal to track any changes or patterns in your health
- Adhere to any prescribed medications or treatment plans

This report is based on a machine-learning model trained on public ECG datasets. Results may be affected by signal quality, lead placement, and patient factors. Always consult a licensed clinician for diagnosis and treatment decisions.